INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Pegylated liposomal doxorubicin · every 4 weeks

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
28 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00205
Version 8

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

Treatment stages

01 **Pegylated liposomal doxorubicin · every 4 weeks**
28-day cycle

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new cough or breathlessness, fever or sudden illness promptly. Tell staff immediately about an infusion reaction or pain at the infusion site. Painful palms / soles or mouth ulcers should be reported before the next dose.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

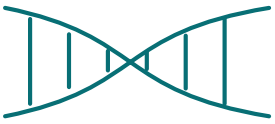
How this medicine works

The aim is to control the cancer and its symptoms. The team reviews benefit, side effects and your priorities before continuing.

LIPOSOMAL CHEMOTHERAPY

Pegylated liposomal doxorubicin

PLD · Caelyx



Damages cancer-cell deoxyribonucleic acid (the cell's genetic material) (DNA)

Doxorubicin is carried inside tiny coated fat particles called liposomes. This changes how it circulates and its side-effect pattern. Heart injury remains possible; skin and mouth effects are especially important.

Liposomal carrier → Doxorubicin reaches cells → DNA damage disrupts division

What to watch for

Report painful palms / soles or mouth ulcers early. Keep heart-test appointments and report breathlessness, swelling or chest symptoms; tell staff immediately about an infusion reaction.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The exact treatment being discussed

Indication

Metastatic breast cancer; advanced ovarian cancer after failed first-line platinum chemotherapy; metastatic soft-tissue sarcoma; or recurrent / advanced endometrial cancer. Eligibility differs by cancer type.

Dose and formulation

50 mg/m². This is the pegylated liposomal formulation (for example Caelyx), not conventional doxorubicin or non-pegylated Myocet. These formulations and doses must not be interchanged.

Evidence limits

Comparable progression-free survival with less cardiotoxicity for PLD, while hand-foot syndrome and oral toxicity were more frequent. Overall survival was similar. Breast-trial findings do not establish efficacy in every NCCP 00205 cross-cancer indication. Normal cardiac function was required in this trial; lower cardiotoxicity does not mean cardiac safety in established heart disease or removal of monitoring. Do not interchange pegylated liposomal, non-pegylated liposomal and conventional doxorubicin.

Why this treatment may be chosen

In a study in metastatic breast cancer, this liposomal formulation controlled cancer about as well as conventional doxorubicin and caused fewer heart problems, but more hand–foot and mouth problems. It can still affect the heart, and personal benefit is uncertain.

The treating team considers the cancer, previous treatment, health and your priorities. Trial averages do not predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

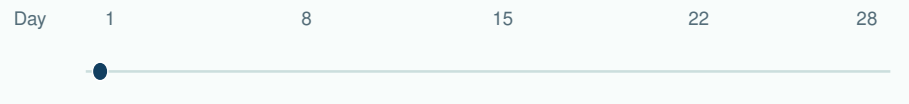
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, reassessment and the next decision

Pegylated liposomal doxorubicin · every 4 weeks

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

28-DAY CYCLE

Pegylated liposomal doxorubicin · every 4 weeks

Repeat according to benefit and tolerance

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

This card describes one active chemotherapy schedule. There is no automatic maintenance phase; continuing, changing or stopping treatment follows clinical review.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Milestones show order, not equal time intervals or booked appointments. The treating team requests tests, reviews results and confirms treatment. full blood count (FBC), renal and liver profile before every 28-day cycle; assess hands/feet, mouth, infection and symptoms. Perform regular electrocardiogram (ECG) monitoring and left ventricular ejection fraction (heart pumping measurement) (LVEF) assessment according to risk and cumulative exposure. LVEF assessment is mandatory before each further PLD administration that exceeds lifetime cumulative anthracycline 450 mg/m²; risk can occur below this threshold.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00205 v8 · official regimen · EMA Caelyx pegylated liposomal product information

THE WHOLE TREATMENT JOURNEY

Pegylated liposomal doxorubicin · every 4 weeks: treatment and checks

Source-defined advanced breast, ovarian, endometrial cancer or soft-tissue sarcoma

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing treatment	Dose change / pause	Next decision
Treatment plan NCCP 00205	Confirm indication, goal and current prescription	Typically 6 cycles or continue according to response and tolerance	Continue according to response, tolerance and prescribed course	Team review before restart or dose change	Agree completion or next treatment
Blood / kidney / liver NCCP 00205 · SMPC	Baseline FBC, kidney and liver tests	FBC, kidney and liver tests before each 28-day cycle	FBC, kidney and liver tests before each 28-day cycle	Recheck recovery and dose before restarting	Follow unresolved abnormalities
Symptoms / medicines NCCP 00205 · SMPC	Review symptoms, medicines and previous treatment	Review effects before each dose	Continue symptom and medicine reviews	Report problems; do not change or restart doses yourself	Agree recovery and next treatment plan
Heart / ECG / echo NCCP 00205 · SMPC	Baseline ECG and echo/ multigated acquisition (heart pumping) scan (MUGA), regular ECG and risk-based LVEF checks throughout treatment; mandatory LVEF before further administration beyond the source cumulative threshold.	Review symptoms and indicated tests	Continue the risk- and treatment-specific plan	Urgent assessment for chest pain, fainting or breathlessnes s	Agree follow-up from exposure and results
Response / course NCCP 00205 · SMPC	Confirm cancer-specific treatment aim	Typically 6 cycles or continue according to response and tolerance	Review benefit, symptoms and the agreed scan plan	Reassess if treatment is held or symptoms change	Agree continuation, change or completion
Bone health / other care NCCP 00205 · SMPC	Review previous/ ongoing treatments and fracture risk	No automatic chemotherapy-cycle dual-energy X-ray absorptiometry (bone density) (DXA)	Keep any separate endocrine/ fracture-risk plan	Review if clinical risks change	Continue separately indicated follow-up

What each check is for

1



Treatment and safety review

Before each planned treatment or four-week review for olaparib

FBC, renal and liver profile before every 28-day cycle; assess hands/feet, mouth, infection and symptoms. Perform regular ECG monitoring and LVEF assessment according to risk and cumulative exposure.

Ask / act: The treating team confirms results and the plan; this map does not authorise a dose.

2 **Benefit and continuation**

At agreed clinical review points

In a study in metastatic breast cancer, this liposomal formulation controlled cancer about as well as conventional doxorubicin and caused fewer heart problems, but more hand–foot and mouth problems. It can still affect the heart, and personal benefit is uncertain.

Ask / act: Discuss benefit, toxicity and priorities before continuing or changing treatment.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your infusion and access plan



STEP 1

Before the first visit

Confirm the indication, baseline bloods, required cardiac checks, medicines review and consent before the first infusion.



STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

The first infusion starts at 1 mg per minute. Later infusions may take 60 minutes if there was no reaction; a previous reaction requires a modified supervised schedule. Staff observe for reactions. Report burning, pain, swelling or leakage at the infusion site immediately.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit length

The first infusion starts at 1 mg per minute. Later infusions may take 60 minutes if there was no reaction; a previous reaction requires a modified supervised schedule. A port is optional access, not a requirement for a take-home pump. This regimen has no routine ambulatory pump.

Premedication

No routine premedicines are specified. Low-risk antiemetic support is separate. Infusion reactions may require a different supervised plan for future doses.

Checks throughout treatment

Ask who arranges each test and who reviews it.

Bloods and symptoms

FBC, renal and liver profile before starting and before each 28-day cycle. Check mouth soreness, palms / soles and recovery from earlier toxicity before each dose.

ECG and heart function

ECG is part of baseline and regular monitoring. The NCCP source advises frequent ECG monitoring; the team should specify when it is due rather than assuming an arbitrary cycle interval. Baseline echo / MUGA is required. Repeat during treatment as clinically indicated, and before each additional administration that exceeds a lifetime cumulative anthracycline exposure of 450 mg/m². Prior anthracyclines count; this threshold is not an automatic treatment stop.

Cancer response

For metastatic breast cancer, ESMO recommends response evaluation generally every 2–4 months, adjusted to disease, treatment and symptoms. Agree the scan type, due window and result-review appointment; symptoms can prompt earlier assessment. For the other cancer indications on this card, use the cancer-specific monitoring plan; the breast interval is not transferred automatically.

Markers

Tumour markers, when useful for the cancer being treated, supplement clinical assessment and imaging. A marker result alone should not determine treatment failure or a treatment change.

How other care fits around chemotherapy

There is no automatic surgery, radiotherapy or maintenance phase in this card.

Surgery and radiation

These are advanced-disease regimens. Surgery or radiotherapy is used only for a defined local or symptom-control purpose. The oncology and radiation teams decide whether chemotherapy continues, pauses or precedes radiation; no routine overlap is implied.

Bone health

dual-energy X-ray absorptiometry (bone density) (DEXA) is not a routine test for this chemotherapy alone. Assess fracture risk and bone density when indicated by endocrine treatment, ovarian suppression or other clinical risks; repeat according to the relevant plan.

Changing treatment

Discuss benefit, adverse effects, other treatment options and your preferences at each review. New symptoms can bring review forward.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Pegylated liposomal doxorubicin · every 4 weeks

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment · Day 1	 **Day 1** intravenous (IV)
Blood counts / infection	 **Days 1–28** Risk varies; fever needs prompt advice
Energy / appetite	 **Days 1–28** May vary and build across cycles
Palms / soles and mouth	 **Days 1–28** Can build across cycles
Heart / breathing symptoms	 **Days 1–28** Report new symptoms promptly

No dates to enter: This chart explains the pattern. The team arranges appointments and tests.

Review milestone: Before every cycle, review mouth / skin recovery and cumulative anthracycline exposure; use the agreed cancer-specific response-assessment plan.

A delay changes the treatment plan: Do not move, double or make up doses yourself. A delayed infusion does not automatically move an already-due scan or heart test.

Why these bands stay broad: Treatment markers show the planned days. Symptom bands show broad possibilities, not measured risk curves or guaranteed symptom-free days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00205 v8 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell needs prompt advice on any day, including planned treatment-free weeks.

Between-treatment notes

Day 1–28: Your reference cycle

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit. The team books actual appointments. Planned breaks are not guaranteed symptom-free days.

Support around the medicines

1 **Supportive medicines**

2 **Everyday care**

Use skin care advised by your team; avoid heat, friction and tight shoes for 4–7 days after infusion. Report painful palms / soles or mouth ulcers early.

3 **Tests and appointments**

Ask who books each test, when it is due and who reviews the result. Tell the team if an expected appointment has not arrived.

4 **Correct medicine and schedule**

5 **Review the full medicine plan**

Low emetic risk with an individual prevention/rescue plan; no routine hypersensitivity premedication is usually required. Reduce heat and friction affecting palms/soles; report painful redness, peeling or mouth ulcers before the next dose. Review cardiac symptoms and lifetime anthracycline exposure; a liposomal formulation still needs cardiac surveillance.

Practical care between visits

Report problems early, including during planned breaks.

Medicine support

Benefit, course and continuing checks

Heart checks and the full course

Baseline ECG and echo/MUGA, regular ECG and risk-based LVEF checks throughout treatment; mandatory LVEF before further administration beyond the source cumulative threshold. LVEF assessment is mandatory before each further PLD administration that exceeds lifetime cumulative anthracycline 450 mg/m²; risk can occur below this threshold.

Questions worth asking

When do I need blood tests, an ECG or echo?

FBC, renal and liver profile before starting and before each 28-day cycle. Check mouth soreness, palms / soles and recovery from earlier toxicity before each dose. ECG is part of baseline and regular monitoring. The NCCP source advises frequent ECG monitoring; the team should specify when it is due rather than assuming an arbitrary cycle interval. Baseline echo / MUGA is required. Repeat during treatment as clinically indicated, and before each additional administration that exceeds a lifetime cumulative anthracycline exposure of 450 mg/m². Prior anthracyclines count; this threshold is not an automatic treatment stop.

Does a port mean a pump at home?

No. A port is a route into a vein. Staff disconnect the infusion and remove the port needle after the visit when appropriate; removal of the implanted device is a separate planned procedure.

> **Try explaining the plan back:** Which days are treatment days, what checks are due before the next dose, and who is arranging and reviewing the next scan?

Pegylated liposomal doxorubicin

- Painful red palms / soles (hand-foot syndrome)
- Mouth soreness or ulcers affecting eating and drinking
- Low blood counts, infection, fatigue and nausea
- Heart damage and infusion reactions; risk depends partly on previous treatment

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Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Pegylated liposomal doxorubicin · every 4 weeks · reviewed supportive care

Active medicines: Pegylated liposomal doxorubicin

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally once	Each treatment day only Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- No routine premedicines are specified. Low-risk antiemetic support is separate. Infusion reactions may require a different supervised plan for future doses.

- This is the pegylated liposomal formulation (for example Caelyx), not conventional doxorubicin or non-pegylated Myocet. These formulations and doses must not be interchanged.
- Assess granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) according to individual risk and the exact source; it is not automatically prescribed by this template.
- Low emetic risk with an individual prevention/rescue plan; no routine hypersensitivity premedication is usually required.
- Reduce heat and friction affecting palms/soles; report painful redness, peeling or mouth ulcers before the next dose.
- Review cardiac symptoms and lifetime anthracycline exposure; a liposomal formulation still needs cardiac surveillance.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

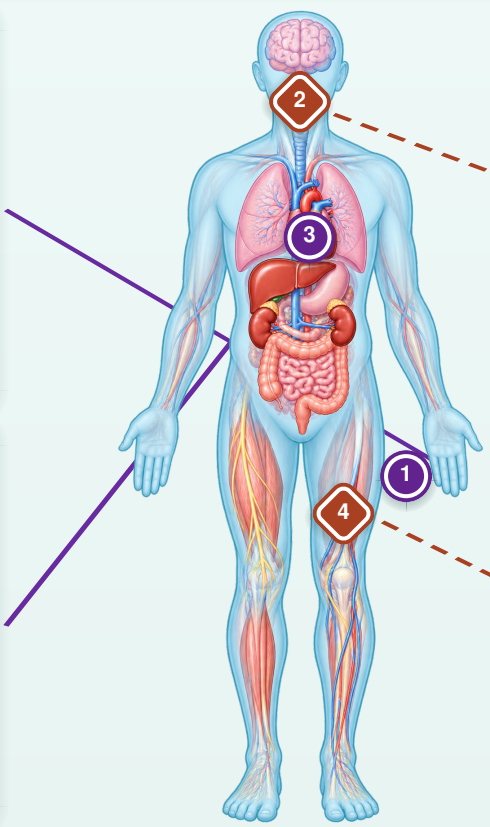
Pegylated liposomal doxorubicin · every 4 weeks

Pegylated liposomal doxorubicin on Day 1 every 28 days, typically for six cycles or until progression or unacceptable toxicity. The team confirms the intended duration and reviews benefit.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Palms / soles

Pegylated liposomal doxorubicin

Report painful redness, swelling or peeling early.

2 Mouth

Pegylated liposomal doxorubicin

Ulcers or difficulty eating / drinking need advice.

3 Heart

Pegylated liposomal doxorubicin

Keep electrocardiogram (ECG) / echo appointments; report breathlessness or swelling.

4 Blood / infection

Pegylated liposomal doxorubicin

Fever, shivers or sudden illness: call immediately.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Selected medicine-specific effects. The full discussion also includes fatigue, hair changes, fertility and other individual risks.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00205 v8 · official regimen**
   https://healthservice.hse.ie/documents/6546/205_V8_Peg_Lipo_DOXO_28_day.pdf
2. **EMA Caelyx pegylated liposomal product information**
   https://www.ema.europa.eu/en/documents/product-information/caelyx-pegylated-liposomal-epar-product-information_en.pdf
3. **O'Brien et al. PLD versus conventional doxorubicin in first-line metastatic breast cancer**
   https://pubmed.ncbi.nlm.nih.gov/14998846/
4. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NICE NG 101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
9. **NCCP NCIS antiemetics v 8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
11. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
12. **ESMO Breast Cancer Pocket Guideline 2023**
    https://data.esmo.org/guidelines/pdf/publicassets/ESMO_2023_BreastCancer.pdf
13. **Cardoso et al. ABC 5 consensus: tumour-marker limitations**
    https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
14. **NCCP antiemetic guidance; reconcile with regimen and individual prescription**
    https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf